



Juneberry Health

The operating system for Care Planning

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Care Plans are Required... But Rarely Delivered



Structured Care
Plans Required

Existing Fee-for-
service billing codes

**<4% Average
Penetration**

Backbone of value-
based care models



\$5 BILLION

In annual reimbursement
tied to care plans that is not
being claimed

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The Solution: Juneberry

An EHR-integrated platform for practitioners and patients to manage care plans that will:

**SAVE
TIME**

for doctors and nurses

**INCREASE
REVENUE**

by leveraging under utilized
care planning billing codes

**IMPROVE
OUTCOMES**

through evidenced-based
care planning &
personalization

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Starting with Alzheimer's and Related Dementias

7.2 Million

People living with AD/ADRD in
the US

\$781 Billion

In Annual Care Costs

Care Planning

25-45% reduced Hospital

Readmissions

Delays Nursing Home Placement

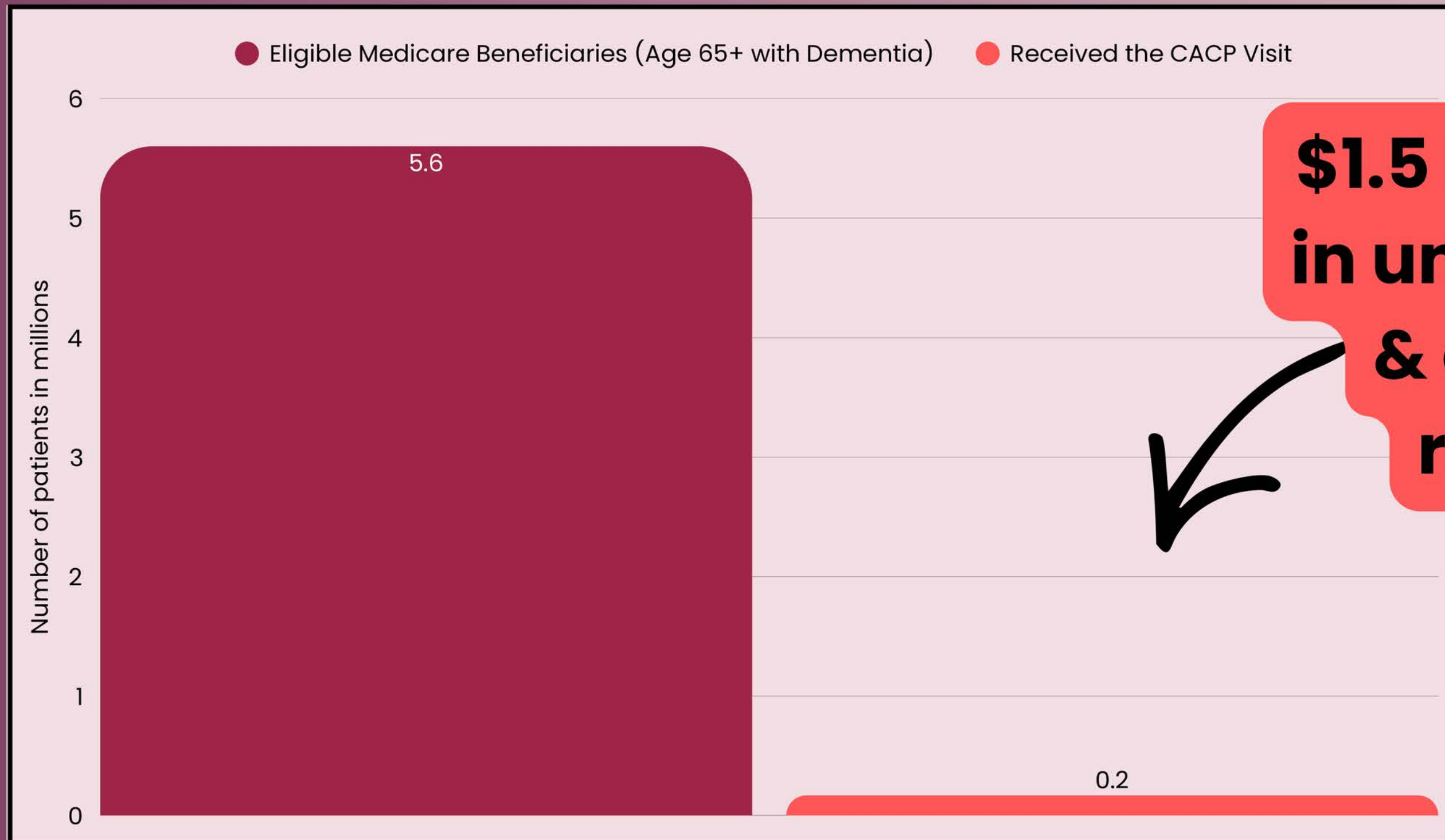
Improves Morbidity & Mortality

→ Saves Money



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CPT 99483 CACP Visit



**\$1.5 Billion (\$280/visit)
in unclaimed Medicare
& other insurance
reimbursement
annually.**

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The Problem: High Burden of Documentation & Short Time



60 Minutes

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Juneberry's TrellisPlan

1. Pre-visit

June intake agent

Conversational AI captures & validates assessment data before the visit:

- Standardized and validated instruments
- Patient + caregiver input

2. In-visit

TrellisPlan engine

63-rule engine + constrained LLM generates a draft care plan

- Auditable clinical logic
- Clinician reviews + signs

3. Post-visit

Actionable outputs

Patient and caregiver get a tailored plan for follow-up and referrals

- Provider care plan
- Patient-friendly plan
- Share visit summaries with care navigators



- Practice submits CPT 99483 for reimbursement (~\$260)
- Practice retains 70%, Juneberry gets 30%
- 2.6X revenue per physician-hour

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Traction

Signed Letters of Support to Pilot Juneberry's Tools



WELLSPAN®

~530 practices
~7900 Providers
~330,000 Eligible
Patients



LIFEBRIDGE
HEALTH.



JOHNS HOPKINS
M E D I C I N E

JOHNS HOPKINS COMMUNITY PHYSICIANS

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Go to Market Strategy: B2B SaaS Business Model



We Sell to Health Systems and Practices; They replace lower revenue visits with higher revenue visits; We take a fraction of the increased revenue

250,000



\$50



\$12.5M

**Patients using
Juneberry**

Avg Per-Visit Fee
~30% of increased
revenue

ARR

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What this means for Juneberry

TAM

\$5 Billion

Uncaptured Care Planning
Revenue from fee-for-
service & Value-Based Care
Programs

SAM

\$28 Million

10% of eligible population

SOM

\$12.5 Million

ARR, by year 5



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Our Team



Emily E. Haroz, PhD
CEO & Founder

Health Services
Researcher; Applied AI &
Successful FDA IDE
experience; \$15+ million
in NIH grants



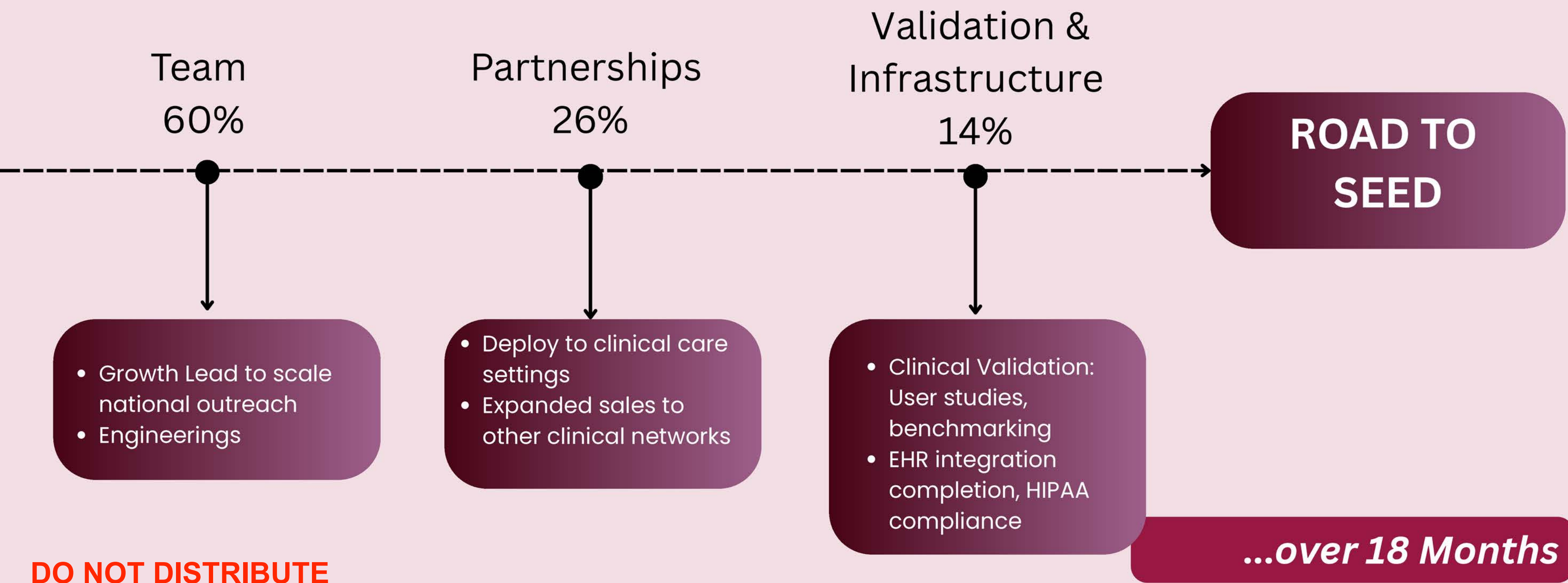
Roy Hodgman
CTO & Founder

Data scientist and
software engineer, 47
AI/ML patents and 20+
years of production
experience

Advised by medical doctors from
Johns Hopkins & seasoned
entrepreneurs with experience
launching & scaling successful startups



Raising (Pre-Seed): Build platform, target dementia care planning | 75,000 patients | \$500k ARR → **REQUEST: \$1.7 Million**





Juneberry Health

Scaling compassionate care planning...

...to improve health and generate revenue

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